BASELINE → TREATMENT → FOLLOW-UP

# Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

| Check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
| **Neuropathy / cold sensitivity** | Record existing nerve symptoms | Check persistence and effect on daily tasks | Report worsening numbness or cold-triggered throat symptoms | Consultant reviews the oxaliplatin component separately | Nerve symptoms can continue after treatment; arrange review |
| **Heart checks** | Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen. | Ask about new chest pain or breathlessness | Chest pain needs urgent assessment | Repeat heart tests if clinically indicated | Follow any heart problem; no fixed echo series |
| **Response / scans** | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00209 v10a · official regimen

THE WHOLE TREATMENT JOURNEY

# Modified FOLFOX-6: treatment and checks

## Gastrointestinal cancers

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** NCCP 00209 | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** NCCP 00209 | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
| **Neuropathy / cold sensitivity** NCCP 00209 | Record existing nerve symptoms | Check persistence and effect on daily tasks | Report worsening numbness or cold-triggered throat symptoms | Consultant reviews the oxaliplatin component separately | Nerve symptoms can continue after treatment; arrange review |
| **Heart checks** NCCP 00209 | Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen. | Ask about new chest pain or breathlessness | Chest pain needs urgent assessment | Repeat heart tests if clinically indicated | Follow any heart problem; no fixed echo series |
| **Response / scans** NCCP 00209 | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

# What each check is for

1 Confirm your personal plan

Before starting

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

**Ask / act:** Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00209

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32